

Thyroid enlargement

NICE NG145 2019, NICE NG12 updated 2023,
BJGP 2019;69:462, BMJ 2020;368:l6670, Thyroid 2016;26:1



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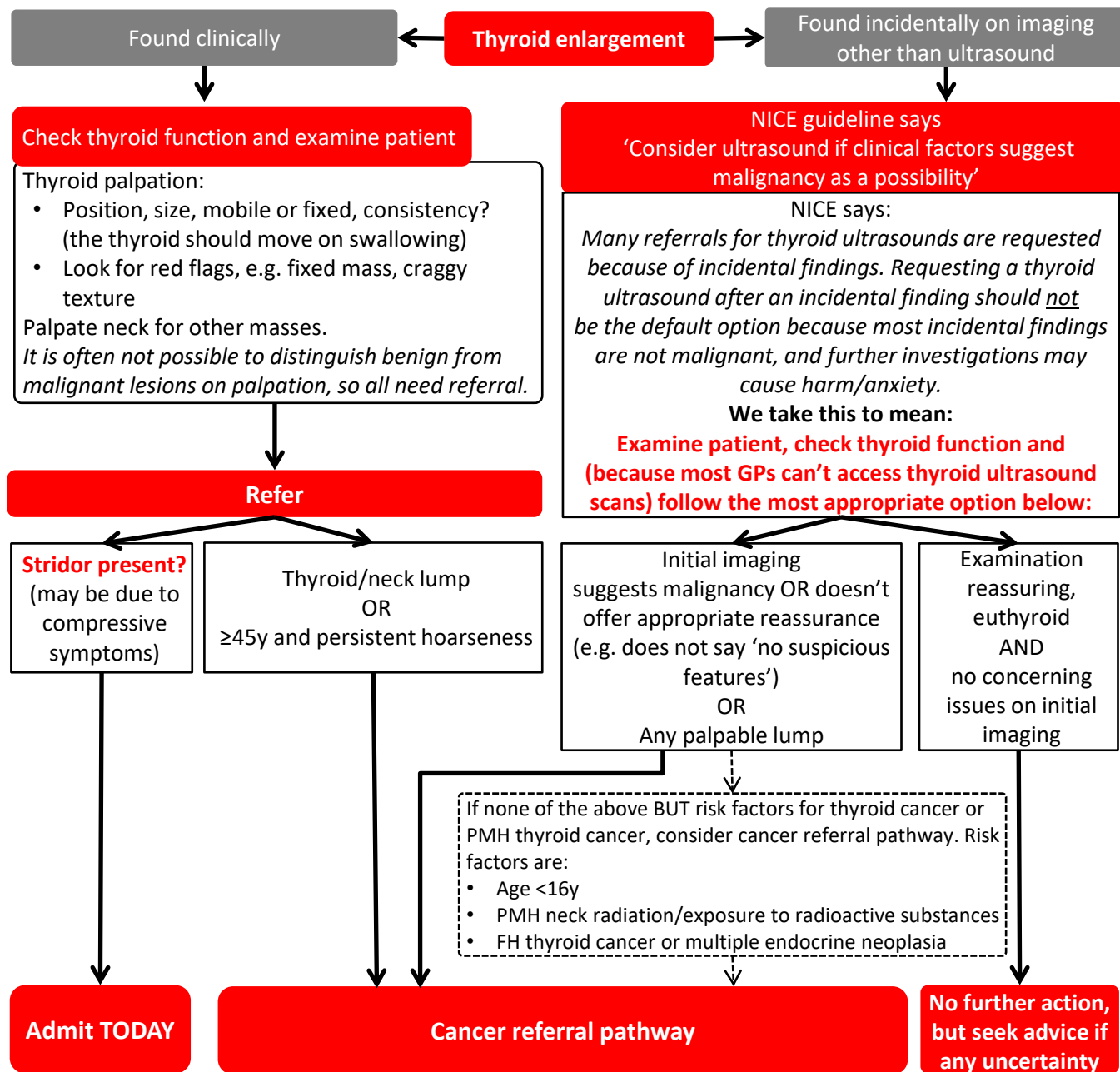
NICE guidance on thyroid enlargement applies to those who are euthyroid. If thyroid enlargement with altered thyroid function, manage their hypo/hyperthyroidism AND REFER, following pathway below, if appropriate.

Differential diagnosis

80% benign: colloid nodule, adenoma, dominant nodule within a multinodular goitre, hyperplastic nodule.

20% malignant: primary thyroid cancer, lymphoma, metastatic disease (e.g. from breast, kidney, prostate)

Most of the time it is not possible to distinguish benign from malignant lesions on palpation, so all need referral



Secondary care will arrange an ultrasound as this is the best test to determine who needs an FNA.

Most with non-malignant enlargement and normal thyroid function need no treatment.

Intervention may be offered if compressive symptoms – be guided by secondary care.

Reassess if clinical picture changes.

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